Which treatment this guide covers NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

NCCP 00565 · VERSION 3

Dacomitinib monotherapy

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea, dehydration and kidney problems

☐ Mouth soreness and reduced appetite

☐ Liver injury

☐ Fatigue and weight loss

☐ Rash, nail changes, dry or peeling skin

☐ Serious lung inflammation

☐ Eye inflammation

Discuss the aim of advanced cancer control, expected benefit and uncertainty, alternatives including not proceeding, bowel and skin effects, serious lung/liver problems, fertility and pregnancy, daily treatment, monitoring and your right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

Confirm the activating EGFR test, first-line treatment setting and prescription. Check blood count, kidneys, liver and bone profile, and arrange baseline staging scans. Review lung symptoms, bowel and skin health, all medicines and pregnancy precautions. NCCP requires regular blood count, kidney, liver and bone tests but does not specify a fixed interval; your team sets appointments. Diarrhoea often starts early, so keep the symptom plan ready from the first dose. Scans follow your Consultant’s plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending